

Therapeutic Relationship

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This article addresses the important role of the therapeutic relationship in cognitive-behavioral therapy (CBT). As has been noted from the inception of CBT, there are critical features of both the therapist and the relationship that optimize the likelihood of therapeutic success, and this article briefly describes these features. It is further argued that a successful therapeutic relationship in CBT is not static but is flexible and adaptive both across and within clients, as their needs and concerns vary. A positive therapeutic relationship is viewed within CBT as a necessary but insufficient condition for change, as the relationship serves as a foundation upon which interventions are scaffolded, but that the client's response to various interventions itself shapes the interactions between therapist and client. Finally, it is argued that the key therapeutic ingredients of CBT are largely teachable, and the article provides several suggestions to promote an effective therapeutic relationship in CBT.

THIS ARTICLE addresses the important role of the therapeutic relationship in cognitive-behavioral therapy (CBT). As has been noted from the inception of CBT and the seminal work of Aaron Beck and colleagues (Beck et al., 1979), there are critical features of both the therapist and the relationship between the therapist and client that optimize the likelihood of therapeutic success. After a brief review of the common features of a positive therapeutic relationship in psychotherapy, this article briefly describes these features as found in CBT and the work of Aaron Beck, in particular.

The therapeutic relationship has been a matter of discussion since the inception of psychotherapy. As early as the work of Sigmund Freud, there was a recognition that although the successful resolution of psychoanalytic therapy related to uncovering unconscious processes, this process rested on a dynamic interplay between the human encounter involved in a therapeutic relationship as well as the judicious use of techniques such as free association or dream analysis (Balint, 1950). Existential psychoanalysis reflected this same recognition:

Apart from ... exceptional circumstances, the two extremes, encounter and technique, seem to be a matter of theoretical importance only. Live practice hovers between the extreme poles. Neither should be looked upon contemptuously or disparagingly. (Frankl, 1961, p. 81)

In somewhat of a contrast to the above arguments, and in a partial response to the developments of behaviorism in the period following the Second World War, Carl Rogers wrote a highly influential article in which he argued that the necessary *and* sufficient conditions for therapeutic change included a congruent therapist, the therapist experiencing unconditional positive regard for the client, the therapist experiencing an empathic understanding of the client, and the communication of these experiences to the client (Rogers, 1957). In a further and more direct expression of these same ideas, he wrote:

The warm, subjective encounter of two persons is more effective in facilitating change than the most precise set of techniques growing out of learning therapy or operant conditioning. (Rogers, 1961, p. 93)

As has been discussed elsewhere, the early development of Beck's cognitive theory and therapy grew primarily from two sources (Dozois et al., 2017). One early influence was the training of Dr. Aaron Beck in psychoanalysis, and its reliance on both relationship and technique in psychotherapy. The other was the use of empiricism and the behavioral tradition, with

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its emphasis on measurable change, and its consequent focus on the methods or techniques that optimize (observable) change. From its inception, cognitive therapy therefore highlighted the synergistic role of the therapeutic relationship *and* the therapy techniques and methods to optimize treatment outcomes. Indeed, the first complete treatment manual based on cognitive therapy (Beck et al., 1979) devoted an entire chapter to the nature of the therapeutic relationship in treating depression. This chapter highlights such key features of a successful cognitive therapist as warmth, accurate empathy, and genuineness, and also elucidates critical aspects of the therapeutic relationship, including trust, rapport, and therapeutic collaboration. Elsewhere in this same volume it is suggested that the therapist can encourage the client to reflect on the issues raised in the therapy sessions, in what can be considered a precursor to what is now referred to as Socratic dialogue (Beck, 2005; Dobson & Dobson, 2017; Kazantzis et al., 2017).

Discussion of the nature of the therapeutic relationship also features in many other books related to cognitive therapy for various disorders, including works by Beck (1988, 1999; Beck & Emery, 1985; Beck et al., 1993; Beck et al., 2008; Beck et al., 2015) and many other books related to CBT more generally (cf. Arntz & Jacob, 2012; Beck, 2021; Dattilio & Freeman, 1994; Dobson & Dobson, 2017; Dugas & Robichaud, 2007; Fairburn, 2008; Segal et al., 2012).

Most of the discussions of the therapeutic relationship in cognitive and cognitive-behavioral therapies converge on three primary themes. The first is that a constructive and successful relationship is integral to positive results with CBT (Okamoto et al., 2019). In this regard, the research literature in the general field of psychotherapy (Norcross & Wampold, 2011) has been highly informative, as it has demonstrated that such features as congruence, authenticity, empathy, positive regard, and a positive alliance/working relationship are important features of successful therapeutic relationships across therapeutic modalities. Within the field of CBT these constructs are often reflected in two primary features of the therapy relationship: collaborative empiricism and the Socratic process (Kazantzis, et al, 2017). The first essential aspect of the therapeutic relationship in CBT is collaborative empiricism; the therapist and client are encouraged to work together as a team to understand and then modify problems through an iterative process of designing assignments that take place in the time between therapy appointments, and to then adjust these assignments and tasks, as the evidence emerges as to what changes are most adaptive in the client's life. The process of Socratic dialogue (see also J. Beck,

2021; Dobson & Dobson, 2017) is a respectful style of interaction, in which the therapist does not assume that they "know best," but rather that they can learn to understand the client based on inductive questions and reflective dialogue, so that the client and therapist together gain an optimal understanding of the issues that have brought the client to therapy, and that can then suggest roads for improvement. Although there are times and circumstances in CBT when the therapist may need to be more confrontive (Linehan, 2014; Young, et al, 2003), this interactive and dialogical style is typically provided, based on the case conceptualization and awareness of what may help the client best reach their treatment goals.

Second, the literature has converged on the importance of the optimal therapy relationship as one that is dynamic and flexible (Castonguay et al., 2018). For example, effective therapists are responsive to different interpersonal aspects and needs of the client across different clients, problems, and diagnoses (see Norcross & Wampold, 2011), remaining adaptable to change over the course of the treatment for a given client or in response to issues such as "therapeutic ruptures." As an example, when working with a more depressed client the CBT therapist may need to provide more structure and encouragement for the client to try ideas in the early stages of treatment, but as the client hopefully responds and becomes less depressed, the effective therapist will invite the client to take more of a coleadership role, and to perhaps design their own homework assignments (Beck et al., 1979). As another example, clients may have had difficult relationships in their past; if so, it would be unrealistic to think that they will not bring relationship concerns or issues into the therapy situation. Thus, effective therapists need to gauge the expectations, social skills, interpersonal styles, and relationship affects that the client brings to the treatment room, and to adjust their style of interaction accordingly.

Third, and perhaps somewhat controversially, I suggest that the data strongly supports the conclusion that a positive therapeutic relationship is a necessary but insufficient condition of therapeutic change in CBT. Indeed, the field of CBT has underscored the critical role of many therapeutic techniques, including but certainly not limited to activity scheduling, exposure to avoided situations and stimuli, skills training, cognitive restructuring, mindfulness training, and the examination of core beliefs in relationship to presenting problems. It is also to my mind incontrovertible that the success of cognitive therapy and CBT relies in part on what happens in the therapy room, but even more so on what happens between sessions, when the client takes the homework assignments and implements

them in their life (Kazantzis et al., 2005, 2016; Kazantzis & Miller, 2021). The precise value of a given technique or homework assignment will depend on a variety of factors, including the presenting problem(s) of the client, the stage of therapy, the skill of the therapist to effectively deploy the technique, the willingness of the client to attempt the method, and of course the outcome of the use of the technique. Successful CBT therapists have a large “toolkit” of techniques that they can go to, to select, adapt, and deploy those interventions that have the highest likelihood to lead to successful client outcomes.

One of the encouraging aspects of CBT is that many aspects of an effective therapeutic relationship can be taught. Psychopathology courses are one strategy to effectively discuss client presentation from a diagnostic perspective, and to encourage the emerging therapist to consider individual differences across clients. Case conceptualization is a fundamental CBT skill (Dobson & Dobson, 2017; Kazantzis et al., 2017) that helps the therapist reflect on the various types of problems that clients can present with, and the moderating factors that influence this presentation. Many of the more explicit skill sets involved in the development of the CBT “tool kit” are also highly teachable. Many excellent manuals exist that describe CBT techniques and how they can be effectively administered. There are workshops on CBT training that occur in many places of the world, and some that are offered virtually and as such are universally available.

As the evidence continues to accumulate, the field of cognitive-behavioral therapy is able to provide more explicit guidance about the critical relationship factors in successful therapy (Kazantzis et al., 2016, 2018). Thus, while there are still open questions about whether any novice therapist can be trained to become a competent CBT therapist, and there are ongoing studies about whether it is more important to be adherent to methods of CBT than to be maximally competent and flexible in the delivery of methods, the extant literature strongly suggests that therapist can learn many of the key elements of the approach. The emergence of training guidelines (e.g., Ashbaugh et al., 2021) and volumes about training (Sudak et al., 2015) strongly suggest that the development of a successful therapeutic relationship is itself amenable to CBT principles.

In summary, this brief article has argued that there has been a long-standing awareness in the field of psychotherapy that a positive therapeutic relationship is pivotal to achieve desired client outcomes. This recognition also can be found in the earliest of the works in the field of cognitive therapy and continues to be emphasized in recent works (cf. Beck, 2021; Dobson

& Dobson, 2017; Kazantzis et al., 2017). Indeed, anyone who had the privilege of seeing Aaron Beck in person engaged in cognitive therapy with a client, or who has viewed any of his training films, can witness his remarkable interpersonal skills, and the ability he had to adapt to the client’s current state and clinical needs. The field of psychotherapy has been enriched by this awareness of the critical role of the interpersonal process, and through both the clinical demonstration of these issues as well as their formalization in theory and research.

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